It's nice to meet you Mr Johnston. You've been referred to me by Dr Bruce. Why don't you tell me why you're here.

M

*Uh… Well… Yeah… So, about a week ago, I was in a **(1) car accident**. Nothing really happened at the time… Um… I spoke to a doctor that day, uh… but he was still a bit concerned. So, uh… a couple of days later, I think it was about that, I was definitely starting to feel different.*

F

What happened when you first went to the doctor?

M

*Well, right after the accident, there was really no problem. I didn't feel anything. It didn't hurt at all. He did say … the doctor I mean… he said that I should watch out, just pay attention, to see if I had a sore neck, or sore back or anything. Um… And that's what happened. Uh… a few days later, I was starting to feel a little bit **(2) dizzy**, um… and then…That's when he said it was definitely **(3) whiplash** and he referred me to you.*

F

Okay. And how have you been feeling since then?

M

*Well, I've definitely had a **(4) sore neck**. It's really been bothering me the last day or so. And, like I said, I've been feeling a bit dizzy. Actually, I have to sit down every once in a while 'cause it gets a little… disruptive, difficult to work and that sort of stuff. I've been feeling a bit **(5) sick** too. I actually had to take yesterday and today off work 'cause I'm finding it quite difficult. I've just been really tired as well.*

F

Is there anything else in your medical history I should be aware of? Any allergies, any other recent visits to the doctor?

M

*I don't think there's anything that pertains to uh… to my… to this… to this injury. I'm allergic to **(6) penicillin**. I just found that out recently. That could have been quite dangerous. And last year, I had the flu really bad… Nothing that really relates to this.*

F

It says here you play hockey.

M

Yeah, yeah, every weekend.

F

Any injuries I should know about?

M

Not really, I've been pretty lucky, actually. A couple of months ago I had a ***(7) sprained ankle****, which is pretty common in hockey. Uhh… we usually we have good trainers though and they keep an eye on us. We take very good care of ourselves.*

F

Nothing serious then.

M

No, I've been pretty lucky.

F

And just take me through your family's medical history.

M

Um… Well, there's ***(8) hypertension*** *and* ***(9) heart disease****. Those are two things that have been there for my grandparents and my parents. My parents are actually taking it quite seriously, so they're eating a lot better these days. I do as well. And my grandfather had a stroke a few years ago. That was pretty hard, and now, he's showing some early signs of* ***(10) Alzheimer's****.*

F

Okay, well, I'd like to examine you in a minute, but first I need to make sure you understand that you're going to have to stop the sports for the moment.

M

Oh, really? That's kind of a tough one. Sports are a pretty big part of my life. I mean, I really don't know how I feel about…like, it's pretty tough for me to sit still. And um… I do have… I mean… we do have this big ***(11) hockey tournament*** *coming up next week. Is there really no way I can play in that? I mean, I can take it easy when I'm playing. There's got to be a way I can get out there. I mean… I've been working*

towards this all year.

F

*Well, I can't stop you from playing but I strongly advise against it because if you get hurt, you can cause permanent serious damage. You really need to rest and (**12) stay off your feet**.*

M

What if I'm feeling okay by next week? Would it be all right to.. you know, to play then? If that was the case, I mean… (FADE OUT)

So, Mrs Wilcox, your GP referred you to me because you've been having a bit of trouble sleeping. Why don't you tell me what's been happening.

F *Okay, yes, that's right. I suppose I've always had some trouble breathing fully through my nose, um… especially **(1) through** my **left nostril**… but I feel like it's been worse in the past few years. I used to play a lot of sport, especially netball, but I've ah… I've had to give it away because I was getting so terribly **(2) short of breath**. So, while it's there during the day, the blockage, ah… it's even worse at night. When it's time for bed, well, sometimes the left side of my nose can feel completely stuffed up. My husband tells me that I've been **(3) snoring** as well. It started a little over 12 months ago, I suppose. Yes, probably 12 to 18 months.*

M *Do you feel tired in the morning?*

F *I do, yes. Um… sometimes it can feel like I've hardly slept at all. And the back of my throat tastes like I've smoked a pack of cigarettes. I've tried some of those sprays, you know, those ones you can get from the chemist, those* ***(4) decongestants****, just to try and help clear my nose a bit. They, ah, used to work, but then after a while they all stopped being effective. Then, when I first spoke to my GP about what was happening she prescribed a spray for me. It's a combination* ***(5) corticosteroid antihistamine spray****. It was wonderful, I really thought it was going to fix it right up. But she said I shouldn't really stay on that long-term because she thought my issues were more structural. That's why she recommended I come and see you.*

M *Okay, great. Do you mind if I take a look. This won't hurt, but it will allow me to confirm your GP's diagnosis.*

F *No, no problem.*

M *Okay, thanks. Well it definitely looks like you have a* ***(6) deviated septum.*** *You have some quite* ***(7) large turbinates*** *as well.*

F *Right, okay. That's what the GP said wasn't it?*

M *Yes. It isn't so unusual – almost no one has a completely straight nose. The issue is that yours is affecting your breathing and that's making it harder for you to sleep.*

F *Hmm, yes, that's right. Can it be fixed then?*

M *It can, yes. The best option is usually a very simple surgical procedure called a* ***(8) septoplasty.***

F *Right, I see…* ***(9) surgery****… I'm not so keen on that. The only other time I had surgery was on my* ***(10) knee*** *and it wasn't very successful. If anything I think it actually made it worse.*

M *Well, septoplasty is very successful and very common. It will allow me to straighten and align the cartilage structure that exists in the midline of your nose and you'll find you'll be able to breathe much more freely after that.*

F *Hmm, I just don't know if that's the way I want to go.*

M *There are other options we can try. Perhaps **(11) less structurally invasive**. I can outline some of those if you like.*

F *Yes, I just don't know. I can suffer with a bit of **(12) anxiety** sometimes, you see. It's ah… it's something I've had to battle with for a while now. I suppose I'm just a little worried that if we ah, if we decide to go ahead with surgery, I might um… I might have some sort of panic attack, you know, on the day itself. Oh, I just don't know what I should do.*

You hear a nurse and a senior nurse discussing a patient.

Where is the patient's range of motion most restricted?

Select one:

- A. in his left shoulder.
- B. in his right knee.
- C. in his hips.

Feedback

Your answer is correct.

Transcript

M *Joe is much better today.*

F *That's good to hear.*

M *Yeah, he's just done his nine o'clock exercise session. He has normal movement in his right shoulder now – he performed both rotation and flexion exercises very well.*

F *What about the hip exercises?*

M *He can do the abduction and the extension exercises but he still has a lot of pain in his right hip. And as for exercises for the left shoulder, he can only do the shoulder flexion to 100 degrees and he can't do the shoulder extension exercises at all for the moment. I'm writing up the flow sheet now. Do you need any other information?*

The correct answer is: in his left shoulder.

Information

Flag question

Information text

Question 2 Audio

Question 2
Incorrect
Mark 0.00 out of 1.00

Flag question

Question text

You hear an optometrist talking about amblyopia to parents and teachers at a kindergarten.

What is he optometrist doing?

Select one:

- A. explaining the effects of the condition.
- B. recommending children get tested.
- C. outlining treatment options.

Feedback

Your answer is incorrect.

Transcript

M *I was asked here today to provide important information regarding amblyopia in children. Amblyopia is a condition in which the nerve pathway from an affected eye to the brain is not properly developed during childhood. It is the most common visual impairment among children and affects up to 3 percent of the young population. However, because only one eye is usually affected and the other eye does the work of both, many parents are unaware of the problem. Fortunately, it can be easily detected with a thorough eye examination and the earlier the condition is detected, the easier it is to treat. Treatment depends on the particular cause of the amblyopia. However, most commonly eye glasses, prisms and eye drops or patches are used to blur the vision of the normal eye and force the brain to recognise the image from the eye with amblyopia. Now the government is offering free eye tests at our center for Medicare Card Holders, so I urge you to take advantage of this offer.*

The correct answer is: recommending children get tested.

Information

Flag question

Information text

Question 3 Audio

Question 3
Correct
Mark 1.00 out of 1.00

Flag question

Question text

You hear a nursing lecturer talking about side effects to a group of student nurses.

What is she discussing?

Select one:

A. the medications with the greatest amount of reactions.

B. the types of patients who experience reactions to medication.

C. the reactions some people experience to some common medications.

Feedback

Your answer is correct.

Transcript

F *Today we're talking about side effects. We'll look at the most common drugs first. As you know, some patients suffer side effects and others don't. And not all patients suffer the same side effects either. Let's consider ibuprofen, for example, a very common pain medication. Some people may suffer dizziness and others might feel nauseous. It's also possible to have diarrhoea, vomiting, even headaches with this type of pain medication. Injecting drugs intravenously may cause swelling at the injection site for some patients. Stomach aches are a side effect of some anti-allergy drugs, as well as loss of appetite and constipation. Patients who take Valium may experience drowsiness, while others may get a rash. Tremors are another side effect of this drug.*

The correct answer is: the reactions some people experience to some common medications.

Information

Flag question

Information text

Question 4 Audio

Question 4
Correct
Mark 1.00 out of 1.00

Flag question

Question text

You hear a senior registrar talking about cholesterol to a group of junior doctors.

What is he doing?

Select one:

A. highlighting the dangers of doing nothing.

B. explaining when treatment may be required.

C. suggesting that it can be difficult to make a diagnosis.

Feedback

Your answer is correct.

Transcript

M *The average total cholesterol level in the UK is 5.5mmol/l for men and 5.6mmol/l for women, which is above a normal level. So does that mean that most people need to take anti-cholesterol drugs? In recent years, we've come to realise that to decide whether an individual's cholesterol levels are dangerous, these levels need to be considered in the light of the person's overall risk of heart disease. This overall risk is determined by a combination of factors, including age, gender, family history of heart disease, and whether someone smokes, is overweight, has high blood pressure or diabetes. The higher the risk of heart disease – for example, a male smoker with high blood pressure and diabetes – the greater the need to get cholesterol levels down.*

The correct answer is: explaining when treatment may be required.

Information

Flag question

Information text

Question 5 Audio

Question 5
Incorrect
Mark 0.00 out of 1.00

Flag question

Question text

You hear an agency nurse checking his schedule with an employee at the agency.

When does the agency employee need the nurse to work?

Select one:

A. on Saturday.

B. on Monday.

C. on Friday.

Feedback

Your answer is incorrect.

Transcript

M *Hi Karen, it's Tyler Brewster. Are you able to give me my schedule for this week please?*

F *Sure. Is the afternoon shift okay for you?*

M *I have exams in the morning, so the afternoon shifts are perfect, yeah.*

F *Okay, so you start on Tuesday at 16.45 and finish at 23:00. On Wednesday you start at 15:30 and finish at 22:15. It's the same on Thursday.*

M *That's good for me.*

F *Are you free on Friday? I need a staff nurse for St Peter's Care Home in Hillsworth.*

M *I'm not free on Friday, no. But Saturday ... I can work on Saturday if you need me.*

F *Hmm ... Saturday... Can you do a shift at the Amazon Hospital?*

M *No problem. What time is the shift?*

The correct answer is: on Friday.

Information

Flag question

Information text

Question 6 Audio

Question 6

Correct

Mark 1.00 out of 1.00

Flag question

Question text

You hear a conversation between a senior nurse and a student nurse.

The senior nurse is

Select one:

A. explaining a range of safety measures.

B. highlighting how easily accidents can happen.

C. outlining the use of some important hospital equipment.

Feedback

Your answer is correct.

Transcript

F *If you give an injection, always throw away the needle in the sharps box here.*

M *What about the cap?*

F *Never recap the needle. Just drop it in the box.*

M *Why?*

F *So you don't injure yourself with the needle. And we have a special place where we throw away the sharps boxes. If you want to throw away other material like gauze or surgical tape, never put them in the sharps box. Sharps boxes are very expensive and we have a special waste room where we throw them away.*

M *Okay, I understand.*

F *And if you take blood, always use disposable gloves. And in the OR, if you pass sharps like needles and knives, never use your hands. Always use a tray…*

The correct answer is: explaining a range of safety measures.

Ms Parr has raised the issue of belly fat because

Select one:

A. more education is required.

B. it is affecting so many people.

C. experts can't agree on its causes.

Feedback

Your answer is correct.

Transcript

My guest today is Eli Parr a dietician and Associate Professor in Nutrition at Deakin University,
M *who has been speaking recently about the dangers of belly fat. So, Ms Parr, why are you raising awareness about this issue?*

Well, Australians are getting fatter and there's no dispute over the fact that this increasing weight is affecting our health. Excess storage of fat is linked to many different chronic diseases. But some areas of fat storage on the body are worse than others. In general, women have greater absolute body fat percentages than men. Typically, women carry more fat around the legs, hips
F *and buttocks, as well as the chest and upper arms. Women have more subcutaneous fat – the fat you can pinch under your skin – while men typically have more visceral fat, which is stored in and around the abdominal organs. People who have greater fat stores around their butt and thigh regions are at lower risk of chronic diseases than those with greater fat stores around their middle.*

The correct answer is: it is affecting so many people.

Question 2
Correct
Mark 1.00 out of 1.00

Flag question

Question text

Why does Ms Parr believe belly fat is so dangerous?

Select one:

- A. Most people have more of it than they used to.
- B. There are currently no medications to treat it.
- C. It causes a complex response in the body.

Feedback

Your answer is correct.

Transcript

M *Why is belly fat dangerous?*

Researchers have found excess visceral fat storage is a significant risk factor for metabolic health complications of obesity such as type 2 diabetes, fatty liver and heart disease. Fat cells in a healthy person are able to grow, recruit inflammatory cells to help reduce inflammation, and
F *remodel themselves in order to allow for healthy body growth. But if there is excess fat tissue, these mechanisms don't function as well. And with excess fat, the body can become resistant to the hormone insulin – which maintains our blood sugar levels. Visceral or belly fat secretes greater levels of chemicals that trigger inflammation – and releases more fatty acids into the*

bloodstream. Whereas the fat cells in the leg region, and the pinchable, subcutaneous layers of fat around the middle, store fatty acids within themselves, rather than pushing them into the circulation.

The correct answer is: It causes a complex response in the body.

Question 3
Correct
Mark 1.00 out of 1.00

Flag question

Question text

What are Ms Parr's views about losing body fat?

Select one:

- A. Overall reduction is the most important factor.
- B. Targeting high risk areas is the best approach.
- C. There are good new methods to help lose weight.

Feedback

Your answer is correct.

Transcript

M *Do you think most people should try and lose fat from anywhere?*

F *A recent weight-loss study that looked at where fat mass was lost found the area of fat loss didn't change the risk factors for heart disease and stroke. The important thing was losing fat. While diet and exercise are unable to specifically target regions of fat deposits, fat mass loss from anywhere can improve risk factors. Online ads might tell you a magic workout machine will reduce fat in one particular area, but we can't target fatty tissue the same way that we can target a specific muscle group. Total loss of fat mass, through a healthy diet and exercise, is the best outcome for overall health and reducing either the symptoms of chronic disease or the risk of developing diseases such as diabetes or heart disease.*

The correct answer is: Overall reduction is the most important factor.

Question 4
Incorrect
Mark 0.00 out of 1.00

Flag question

Question text

Ms Parr considers a healthy diet to be one that

Select one:

- A. avoids all types of sugar.
- B. encourages burning food as fuel.
- C. contains all the bodies' essential nutrients.

Feedback

Your answer is incorrect.

Transcript

M *What do you mean when you say, a healthy diet?*

F *If we look at just two key elements: carbohydrates and fructose. Carbohydrates provide a quick-energy supply that's easy for the body to use, but the body and brain don't need carbohydrates. Your body does need glucose, which can be obtained from carbohydrates, but glucose can also be obtained when it's converted from protein and fat. This means, the lower your carbohydrates intake, the more the body uses fat for energy. Fructose, found in fruit, is something you should definitely avoid if you want to help your body get rid of fat. It's true that fruit can be good for your health (fibre, vitamins) but to lose fat from around your body it's important to limit the intake of fruit. Fructose, activates an enzyme, which also activates other enzymes causing the body to accumulate fat.*

The correct answer is: encourages burning food as fuel.

Question 5

Incorrect

Mark 0.00 out of 1.00

Flag question

Question text

Ms Parr says that some people have trouble losing fat because of

Select one:

- A. their failure to accept they have a problem.
- B. social factors that are difficult to control.
- C. the pressures associated with a busy lifestyle.

Feedback

Your answer is incorrect.

Transcript

M *What about those who do eat well, but still can't lose fat from around their belly, or anywhere else?*

F *Well, if you sleep less than six hours a night – that can certainly be a contributing factor. There was a study done over a 16-year period that took into account almost 70,000 women. The study found that those who slept five hours or less a night were 30% more likely to gain 13 or more kilos than those who slept 7 hours. Stress is another issue. The stress hormone cortisol has been known to increase the amount of fat a person's body maintains and to also enlarge your fat cells. Higher levels of cortisol have also been linked more specifically to increased visceral fat. Age is also an issue. As you get older, your body changes how it gains and loses weight. Both men and women experience a declining metabolic rate. In addition to that, women have to deal with menopause, and if a woman gains weight after menopause, it's usually more likely to be in their bellies.*

The correct answer is: social factors that are difficult to control.

Question 6
Correct
Mark 1.00 out of 1.00

Flag question

Question text

When does Ms Parr recommend people seek further advice?

Select one:

- A. If they are a lot heavier than they used to be.
- B. If they aren't as strong as they once were.
- C. If they notice they have a wide waistline.

Feedback

Your answer is correct.

Transcript

M *And what should people do if they're worried about the amount of fat they have around their belly?*

F *Well, remember, not all fat is the same. And some overweight and obese people with a store of excess fat around the middle of the body might also have more muscle mass, which could be protective. However, by the same token, if you have fat around your belly and it's greater than the size of your hips, you should visit your doctor to assess your cardiovascular health and fat distribution.*

The correct answer is: If they notice they have a wide waistline.

Why does Dr Richter mention the numbers related to deaths from suicide?

Select one:

A. to suggest that better prevention techniques are needed.

B. to indicate where most of the problem is located.

C. to highlight how many people it affects.

Feedback

Your answer is incorrect.

Transcript

Hi. My name is Dr Greg Richter. I'm a primary care physician, researcher and author. My topic today is secondary prevention of suicide. Suicide poses major threats to public health worldwide. In 2002, suicide accounted for about 30,000 deaths in the US alone and almost 900,000 deaths worldwide. This translates to 1.5% of all deaths related to disease. However, I am convinced that these figures can and should be reduced. According to recent research, suicidal behaviour can be distinguished into 4 distinct categories: completed suicide where a person has been successful in their attempt to commit suicide; suicide attempt where a person has been unsuccessful in their attempt to commit suicide; suicidal ideation which is where a person is having suicidal thoughts; self-harm behaviour where a person injures themselves deliberately. It is important to realise that 83% of people who commit suicide or attempt suicide have had contact with a doctor within a year of their death and up to 66% of people who commit or attempt suicide have had such contact within a month of their death.

The correct answer is: to suggest that better prevention techniques are needed.

Question 2

Correct

Mark 1.00 out of 1.00

Flag question

Question text

Dr Richter believes the most important area for suicide prevention is

Select one:

A. understanding where a patient falls within specific classifications.

B. the doctor's role in judging a patient's particular condition.

C. the patient taking responsibility for their current predicament.

Feedback

Your answer is correct.

Transcript

M *We can divide suicide prevention into 3 categories which are primary, secondary, or tertiary. Primary suicide prevention aims to reduce the number of new cases of suicide in the general population. Secondary suicide prevention aims to decrease the likelihood of a suicide attempt in high-risk patients. Tertiary suicide prevention occurs in response to completed suicides and attempts to diminish suicide in a particular geographical area or in a particular age group. The clinical evaluation of the medical and psychiatric history of a patient and of their current state is the crucial and essential element of the suicide assessment process. It is important because it enables the identification of risk factors, in order to assess the patient's immediate safety and the best setting for treatment, and also to develop diagnosis and treatment strategies.*

The correct answer is: the doctor's role in judging a patient's particular condition.

Question 3
Incorrect
Mark 0.00 out of 1.00

Flag question

Question text

What does Dr Richter say helps when identifying suicidal risk?

Select one:

- A. Considering specific views from the patient themselves.
- B. Acknowledging a patient's potential mental health issues.
- C. Accepting that each patient has their own unique problems.

Feedback

Your answer is incorrect.

Transcript

M *Psychiatric illness is a major contributing factor to suicide risk, with mood disorders such as major depressive disorder and bipolar disorder being associated with about 60% of suicides. Indeed, psychiatric disorders are diagnosed in more than 90% of completed suicides, and more than 80% of these psychiatric disorders are untreated. Thus, the recognition and treatment of individuals with psychiatric disorders, specifically mood disorders, are essential components of secondary suicide prevention. In addition, the subjective rating of the severity of depression is one of the most powerful predictors of future suicidal acts. Therefore, assessing and managing depression as well as being aware of the suicide risks in psychologically, medically, and neurologically disordered individuals is an important aspect of secondary suicide prevention. Consequently, physicians need to be taught to recognise the association between subjective findings and suicide.*

The correct answer is: Considering specific views from the patient themselves.

Question 4
Incorrect
Mark 0.00 out of 1.00

Flag question

Question text

What are Dr Richter's views on antidepressants being used in suicide prevention?

Select one:

A. they should only be used with older patients.

B. they are not as well understood as they should be.

C. they need to be more effectively integrated with other actions.

Feedback

Your answer is incorrect.

Transcript

Antidepressant medications are the most widely used pharmacological interventions in secondary suicide prevention, but studies of their effectiveness in reducing suicide attempts have had mixed results. Among adults younger than 25 the effect of antidepressants seems to be neutral on suicidal behaviour but it **M** *seems to reduce the risk of suicidal behaviour in the elderly. Thus, the relationship between antidepressants and suicide needs further studies before this class of drugs can be safely used for the secondary prevention of suicide. Better psychological and pharmacological treatment of depression and alcoholism, also appears to decrease suicide rates.*

The correct answer is: they need to be more effectively integrated with other actions.

Question 5

Incorrect

Mark 0.00 out of 1.00

Flag question

Question text

Regarding the risk of suicide, Dr Richter suggests that

Select one:

A. the need for increased levels of medication.

B. governments could be doing a lot more to help.

C. modern-day society is a major contributing factor.

Feedback

Your answer is incorrect.

Transcript

M *Many studies show that suicides by particular methods, for example, firearms, domestic gas, or pesticides decrease after the introduction of legal restrictions that reduce access to such means. This reduction in suicide rates is particularly influential in regions where the specific means of restriction correlates with a common method of suicide. For example, in the UK the reduction of carbon monoxide in domestic gas since 1958 and the reduced availability of analgesics since the mid-1990s have both decreased UK suicide rates. Many studies have also exposed a need for a decrease in reporting of suicide and for responsible reporting. Media black-outs in reporting suicide have coincided with a decrease in suicide rates, because reports of suicide in the media tend to glamorise suicide for vulnerable individuals. However, the Internet is of increasing concern, with blogs and chat rooms providing accessible instructions for suicide.*

The correct answer is: modern-day society is a major contributing factor.

Question 6

Incorrect

Mark 0.00 out of 1.00

Flag question

Question text

Dr Richter explains that to improve secondary suicide prevention in the future

Select one:

- A. there needs to be attention given to innovative methods.
- B. greater emphasis needs to be placed on physician training.
- C. detailed analysis of what does and doesn't work needs to occur.

Feedback

Your answer is incorrect.

Transcript

M *Despite our increasing knowledge about secondary suicide prevention, there are still many gaps in the research, and looking to the future, thorough evaluations and appropriate treatments of patients with depressive disorders and other psychiatric illnesses should help to improve the efficacy of secondary prevention of suicide. But, it is also clear that more research into new approaches for the prevention and treatment of suicidal behaviour remains essential.*

The correct answer is: there needs to be attention given to innovative methods.